

Vaginal Bleeding During Early Pregnancy

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During the first 20 weeks of pregnancy, 20 to 30% of women have vaginal bleeding. For many of these women, there is a problem with the pregnancy, but some women have light bleeding or spotting and have a normal, healthy pregnancy. In about half of women with bleeding, the pregnancy ends in a **miscarriage**. If miscarriage does not occur immediately, problems later in the pregnancy are possible. For example, the baby's birth weight may be low, the pregnancy may result in a **stillbirth** (death of a fetus after 20 weeks of pregnancy), or the baby may be born early (**preterm birth**) or die during or shortly after birth. If there is heavy bleeding, blood pressure may become dangerously low, resulting in **shock**. However, many women with light bleeding in early pregnancy go on to have a healthy pregnancy and delivery.

The amount of bleeding can range from spots of blood to heavy bleeding. Passing large amounts of blood is always a concern, but spotting or mild bleeding may also indicate a serious disorder. Women with any vaginal bleeding should contact their doctor.

Causes of Vaginal Bleeding in Early Pregnancy

Vaginal bleeding during early pregnancy may result from disorders related to the pregnancy (obstetric) or not (see table **Some Causes and Features of Vaginal Bleeding During Early Pregnancy**).

The **most common cause** of vaginal bleeding during early pregnancy is

- A **miscarriage** (also called spontaneous abortion)

Vaginal bleeding may mean that a miscarriage is possible, but sometimes the bleeding stops and the pregnancy continues without any problems. When a miscarriage does occur, it usually happens in a few stages. First, there is bleeding. Then cramping

develops, which may be severe and the bleeding may become heavy. The woman then usually passes the pregnancy, and she may see pregnancy tissue pass out of the vagina.

Sometimes, a miscarriage does not cause any symptoms and is found when the fetus is assessed during a regular medical visit (called a missed abortion). If a woman suspects she is having a miscarriage, she should contact her doctor. Sometimes women can go through a miscarriage at home, but if the pain or bleeding is severe, or if the pregnancy tissue is not passed completely, they need treatment with medications or a procedure to make sure they are safe and the pregnancy passes completely.

The contents of the uterus may be infected before, during, or after the miscarriage (called a [septic abortion](#)).

The **most dangerous cause** of vaginal bleeding during early pregnancy is

- An abnormally located pregnancy ([ectopic pregnancy](#))—one that is not in its usual place in the uterus—for example, one that is in a fallopian tube

An ectopic pregnancy is life threatening. A pregnancy implanted outside the uterus will not develop normally. As the pregnancy grows in a fallopian tube or other body part (such as the ovary, cervix, old cesarean scar in the uterus, or abdomen), it may cause that part to burst. This leads to severe pain and heavy bleeding, which can result in severe harm or even death to the pregnant woman. When an ectopic pregnancy is diagnosed, the treatment is to end the pregnancy with medications or remove it with surgery.

Another possibly dangerous but less common cause is rupture of a corpus luteum cyst. After an egg is released from the ovary, the structure that released it (the corpus luteum) may fill with fluid or blood instead of breaking down and disappearing as it usually does.

Risk factors

For **miscarriage**, risk factors include the following:

- Age over 35
- One or more miscarriages in previous pregnancies
- [Cigarette smoking](#)
- Use of illicit drugs, such as [cocaine](#), or possibly other substances, such as [alcohol](#)

- Abnormalities in the uterus, such as [fibroids](#), scarring, or an abnormal shape of the uterus

For **ectopic pregnancy**, risk factors include

- A previous ectopic pregnancy (the most important risk factor)
- Previous abdominal surgery, especially for permanent sterilization (tubal ligation)
- Fallopian tube abnormalities
- Current use of an [intrauterine device](#) (IUD)
- In vitro fertilization to conceive the current pregnancy

Additional risk factors for ectopic pregnancy include history of [sexually transmitted infection](#) or [pelvic inflammatory disease](#), current use of estrogen/progestin oral contraceptives, cigarette smoking, [infertility](#), and prior [miscarriage](#) or [abortion](#).

Evaluation of Vaginal Bleeding in Early Pregnancy

Doctors first determine whether the cause of vaginal bleeding is an [ectopic pregnancy](#).

Warning signs

In pregnant women with vaginal bleeding during early pregnancy, the following symptoms are cause for concern:

- Fainting, light-headedness, or a rapid heart rate—symptoms that suggest very low blood pressure
- Loss of large amounts of blood or blood that contains tissue or large clots
- Severe abdominal pain that worsens when the woman moves or changes positions
- Fever, chills, and a [vaginal discharge](#) that contains pus mixed with the blood

When to see a doctor

Women with warning signs should see a doctor immediately.

Women without warning signs should see a doctor within 48 to 72 hours.

What the doctor does

Doctors ask about the symptoms and medical history (including past pregnancies, miscarriages, abortions, and risk factors for ectopic pregnancy and miscarriage). Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause and the tests that may need to be done (see table [Some Causes and Features of Vaginal Bleeding During Early Pregnancy](#))

Doctors ask about the bleeding:

- How severe it is (for example, how many pads are used or soaked in an hour)
- Whether clots or tissue were passed
- Whether pain accompanies the bleeding

If the woman has pain in addition to vaginal bleeding, doctors ask when and how it started, where it occurs, how long it lasts, whether it is sharp or dull, and whether it is constant or comes and goes.

During the physical examination, doctors first check for fever and signs of substantial blood loss, such as a rapid heart rate and low blood pressure. They then do a pelvic examination, checking to see whether the cervix (the lower part of the uterus) has started to open (dilate), which may allow the pregnancy to pass through. If any tissue (possibly from a miscarriage) is detected, it is removed and sent to a laboratory to be analyzed.

Doctors also gently press on the abdomen to see whether it is tender when touched.



Some Causes and Features of Vaginal Bleeding During Early Pregnancy

Cause	Common Features*	Diagnostic Approach†
		Sometimes a doctor's examination alone
		Usually tests as for ectopic pregnancy
A <u>miscarriage</u> that	Vaginal bleeding, sometimes with passage of tissue from the pregnancy	Monitoring for fetal heart rate auscultation
• Has occurred or is occurring		A blood test to measure a hormone produced by the placenta (human chorionic gonadotropin, or hCG)
• May occur (threatened abortion)	Crampy pain in the pelvis and throughout the abdomen	Complete blood count
		Ultrasonography of the pelvis
	Vaginal bleeding, with or without pelvic or abdominal pain, that may be dull and come and go or may be severe, sudden, and constant	A pregnancy blood test
An <u>ectopic pregnancy</u> (an abnormally located pregnancy— not in its usual place in the uterus)	If the ectopic pregnancy has ruptured, fainting, light-headedness, or a rapid heart rate	Ultrasonography of the pelvis
		Sometimes laparoscopy (insertion of a viewing tube through an incision in the abdomen) or laparotomy (surgery involving an incision into the abdomen)
A <u>molar pregnancy</u> (abnormal placental growth with or without a fetus due to	Vaginal bleeding, with or without crampy abdominal pain	Tests as for miscarriage
		Blood tests



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